

We help a daughter sharing her mother's care with her brother and sister,
who never knows what was given this morning, who has Ammi this
afternoon, or what the doctor said on Wednesday,
by giving the three of them one private record that writes the handoff for
them.

Ammi is 78. Fatima is one of three.

Fatima · 07:52

did she take the morning ones?

Sister A · 08:10

yes. she ate half. who has her this afternoon?

Abdullah · 08:41

I thought Fatima. I have the pharmacy at 11 I think?

Fatima · 09:15

no that was last week. what did Dr Rahman say
wednesday??

Sister A · 09:20

scroll up

Three children. Three phones. One group chat.

Nobody knows what she took this morning.

Nobody knows who has her this afternoon.

She is not the user. Her family's mental load is.

A chat is a conversation.

Care needs a record: the plan, what was actually done, the handoff, who carried the week. Reminders and task lists do one piece. We built the record.

The elder is the reason, not the user.

Ammi does not tap. Her children do. She gets her own big-type screen, her wishes on every card, and the truth about who can read her record.

People only write the truth where nobody else can read it.

"She refused her meds and was confused" never goes in a shared app. So the server cannot read ours. Privacy is not a feature here. It is what makes the record honest.

WHY IT IS WORTH SOLVING

1 in 4

American Muslims already care for an older adult. Twice the general public. ISPU, Support and Sacrifice, 2026

1 in 4

family caregivers say they cannot coordinate the care. Up from 1 in 5. AARP / NAC, Caregiving in the U.S., 2020

49%

of family caregivers of people with dementia report burden; 31% depression. Collins & Kishita, 2020, meta-analysis

What changes when the record exists

- A missed meal or dose is seen the same day, not at the next visit.
- The load becomes visible, so it can be shared before someone burns out.
- The ER gets fourteen days on one page instead of a family's memory.
- Less to remember, less to ask, less to repeat.

How we will know

- Plan adherence per day.
- Minutes from handoff to acceptance.
- Share of the week carried by the busiest person.
- Families still logging after four weeks.

Measured in the five-family pilot, Monday.

LIVE DEMO · TWO MINUTES · THREE PHONES, ONE FAMILY

1. **Sister A:** Home. Ammi's day, 10 of 13 done, who has her, next pickup, the recommendations.
2. Log two things at once. Undo one. It stays, struck through.
3. Hand off to **Fatima**. What happened and what is next already written. Send.
4. **Fatima's phone:** the alert, the card, her preferences on top. Accept.
5. **Ammi's screen:** "Fatima is looking after you today." She taps I need help. Every phone goes red. The hospital sheet opens.
6. **Server view.** Same screen, as we see it. Not one word.

Watch for

- Nothing typed except one line.
- Live across phones, no refresh.
- Readable on the phones, unreadable on the server.
- Nurse Layla, blue: one login, two families.

Monday: five families. Then the people who already do this work.

Next four weeks	Who we pull in	How it pays for itself
• Five Montreal families, Urdu and Arabic speaking, one month. • Key recovery between two relatives. Push notifications. • Urdu and Arabic on Ammi's screen.	• A geriatric nurse to review every recommendation. • One EÉSAD home-care agency for a worker-login pilot. • Masjid welfare committees as the family channel. • CLSC and Islamic Relief Canada's network, once the pilot has numbers.	• Free for families, always. No paywall between siblings. • Agencies and care organisations: \$8 to \$15 per elder a month. • The server, about \$1,000 a month, is covered by 84 elders. Estimates, in the backup.

Care was already happening. Now it has a record. And only the family can read it. We ask for introductions, five families, and one agency.